



## Procedure

# Recognising and Responding to Acute Deterioration

## Scope

| Site     | Service/Department/Unit | Disciplines                                           |
|----------|-------------------------|-------------------------------------------------------|
| SCGOPHCG | Organisation Wide       | All Staff, Medical, Nursing, Allied Health, Pharmacy, |

## Contents

|                                                                           |    |
|---------------------------------------------------------------------------|----|
| Scope.....                                                                | 1  |
| Introduction .....                                                        | 2  |
| Risk Statement .....                                                      | 2  |
| Definitions .....                                                         | 3  |
| Governance .....                                                          | 3  |
| Risk Assessment .....                                                     | 4  |
| 1. Procedure.....                                                         | 4  |
| 1.1 Observation and Response Chart.....                                   | 4  |
| 1.2 Minimum Core Physiological Observations .....                         | 5  |
| 1.3 Measurement of Observations .....                                     | 5  |
| 1.4 Frequency of Observations .....                                       | 5  |
| 2. Escalating Care.....                                                   | 6  |
| 2.1 Acute physiological deterioration .....                               | 6  |
| 2.2 Acute mental state deterioration .....                                | 6  |
| 2.3 Physiological deterioration of a mental health patient .....          | 7  |
| 2.4 Delirium and cognitive impairment .....                               | 7  |
| 3. Modifications and Clinical Management Plan for Altered Parameters..... | 8  |
| 3.1 Clinical management plan for altered parameters .....                 | 8  |
| 3.2 Time limited post procedure modifications .....                       | 8  |
| 4. Responding to deterioration .....                                      | 8  |
| 4.1 Escalation of care .....                                              | 9  |
| 4.2 Escalation criteria and action required .....                         | 10 |
| 5. Patient, Family and Carer Decision-making .....                        | 10 |
| 5.1 Communication with patient, family and or carer.....                  | 11 |
| 5.2 Older person.....                                                     | 11 |

|     |                                                                   |    |
|-----|-------------------------------------------------------------------|----|
| 5.3 | Advance Health Directives (AHD) and Advance Care Plans (ACP)..... | 12 |
| 5.4 | Goals of Patient Care (GoPC).....                                 | 12 |
| 5.5 | Aishwarya's Care Call.....                                        | 12 |
| 6.  | Medical Emergency Escalation Criteria.....                        | 13 |
| 6.1 | Medical Emergency Team (MET).....                                 | 13 |
| 6.2 | MET record.....                                                   | 14 |
| 6.3 | Roles and responsibilities .....                                  | 14 |
| 7.  | Education and Training .....                                      | 15 |
| 8.  | Code Black Personal Threat .....                                  | 17 |
| 9.  | Clinical Incident Management .....                                | 17 |
|     | Compliance, monitoring and evaluation .....                       | 17 |
|     | Appendix 1: Mental State Assessment Descriptions.....             | 19 |

## Introduction

This procedure outlines the minimum mandatory requirements of recognising and responding to acute physiological and mental state deterioration for all patients receiving care across Sir Charles Gairdner Hospital (SCGH) and Osborne Park Hospital (OPH).

Many patients who experience serious adverse events along with deterioration of their condition, such as unexpected death and cardiac arrest, often exhibit warning signs (including abnormal vital signs) in the hours prior to the event. Early recognition and response to acute deterioration may minimise adverse events and decrease sudden and unexpected deaths.

Other serious events, such as suicide and aggression, are also often preceded by observed or reported changes in a person's behaviour or mood that can indicate deterioration in their mental state.<sup>1</sup> This guideline pertains to adult inpatients inclusive of mental health and aged care residents within Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG) facilities.

## Risk Statement

Non-compliance with this procedure may result in a breach in mandatory policies, failure to meet the expected standards, harm to patients, their families, and carers, and/or damage to the reputation and image of SCGOPHCG.

|                                       |                                     |                                   |                                     |
|---------------------------------------|-------------------------------------|-----------------------------------|-------------------------------------|
| Breach legislative requirements       | <input type="checkbox"/>            | Impact on Patient Quality of Care | <input checked="" type="checkbox"/> |
| Breach National/State/Hospital Policy | <input checked="" type="checkbox"/> | Impact on Patient Safety          | <input checked="" type="checkbox"/> |
| Breach professional standards         | <input checked="" type="checkbox"/> | Misconduct                        | <input type="checkbox"/>            |
| Breach SCGOPHCG Mission & Values      | <input checked="" type="checkbox"/> | Staff Safety                      | <input checked="" type="checkbox"/> |
| Other:                                | <input type="checkbox"/>            |                                   |                                     |

## Definitions

|                                      |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |
|--------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Acute deterioration</b>           | Physiological or mental state changes that may indicate a decline of the patient's health status.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |
| <b>Community-based services</b>      | For this procedure, community-based services refer to services provided by SCGOPHCG in a community setting.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
| <b>Mental State</b>                  | Refers to a person's intellectual capacity, emotional state and general mental health based on clinical observations and interviewing. Mental state comprises mood, behaviour, orientation, judgement, memory, problem-solving ability and contact with reality.                                                                                                                                                                                                                                                                                                                                                               |
| <b>Deterioration in mental state</b> | A negative change in a patient's mood or thinking, marked by a change in behaviour, cognitive function, perception, or emotional state. Change can be gradual or acute; it can be observed by health professionals, reported by the patient themselves, or reported by their family or carers. Deterioration in mental state can relate to several predisposing or precipitating factors, including mental illness, psychological or existential stress, physiological changes, cognitive impairment (including delirium), intoxication, withdrawal from substances, and emotional response to social context and environment. |
| <b>Clinical Incident</b>             | A clinical incident is an event or circumstance resulting from health care which could have or did lead to unintended and/or unnecessary harm to a patient/consumer.                                                                                                                                                                                                                                                                                                                                                                                                                                                           |

## Governance

The SCGOPHCG Recognising and Responding Acute Deterioration (RRAD) Committee is responsible to the SCGOPHCG Clinical Governance Committee, ensuring alignment with the WA Health MP 0086/18 Recognising and Responding to Acute Deterioration Policy and the National Safety and Quality Health Service: Recognition and Response to Acute Deterioration Standard.

SCGOPHCG has systems in place to support and promote recognition of, and response to, all patients who may be experiencing acute deterioration related to a physiological or mental state, including medical, surgical, mental health patients, and those receiving end of life care.

The RRAD Committee provides local governance of NMHS endorsed acute deterioration systems and processes, and maintains oversight of Observation and Response Charts (ORC), including:

- amendments to existing charts and trigger tools,
- introduction of new charts and trigger tools,
- submission of the ORC Request Form when making a request for changes to an existing ORC or creation of a new ORC.
- evaluation of the endorsed ORC

Oversight over the clinical pathways, tools and processes relating to:

- Acute Anaphylaxis
- Sepsis recognition and response
- Deterioration in Mental State including Delirium
- Goals of Patient Care (GoPC)
- Advance Care Planning
- Aishwarya's CARE Call

The governance committee will be responsible for:

- Regular review of the rapid response system that includes the attendance of appropriately trained health professionals working within their clinical scope with the capability to address physiological and mental state deterioration, 24 hours per day, 7 days per week.
- ensuring clinical staff are aware of and adhere to GoPC and any form of advance care planning documents when managing or escalating care of a deteriorating patient.
- regular review of the effectiveness of the rapid response systems including identified key performance and outcome indicators.
- monitoring and ensuring local compliance with this Procedure and associated site-specific procedures / clinical practice guidelines.

## **Risk Assessment**

On admission or presentation, a comprehensive patient assessment should confirm baseline vital signs, mental and cognitive state so that changes can be recognised.

- Clinical staff should engage with the patient, their family, and carers, where culturally and developmentally appropriate, to identify specific factors that could precipitate deterioration, as well as factors that contribute to the patient's wellbeing.
- Family and carers' information is essential to clarify if fluctuations in level of alertness, mental state, behaviour or if cognitive functions are different from baseline and ascertain if there has been any decline.
- Screening tools are used to assess risks and provide guidance for the management of mental state and mental health deterioration.

## **1. Procedure**

### **1.1 Observation and Response Chart**

- The SCGOPHCG Observation Response Chart (ORC) is a multi-tiered single parameter trigger chart with a mandatory escalation protocol. The escalation protocol states response criteria and/or actions required for the care of all inpatients within general wards/specialty high dependency units (HDUs) and departments.
- Clinicians must respond to abnormal physiological observations by following the escalation and action required at all times unless appropriate modifications are in place.
- The ORC is not used in the Intensive Care Unit or Emergency Department for documentation of routine observations. Prior to transfer from these areas to general wards, observations are to be recorded on the ORC and all escalation criteria is to be followed.
- Theatre and procedural departments are not required to document intra-procedural observations on the ORC, although the last set of observations prior to transfer to an inpatient ward are to be documented on the ORC and the escalation criteria followed.

## 1.2 Minimum Core Physiological Observations

The six **core physiological observations** are:

- Respiratory rate
- Oxygen saturation
- Heart rate
- Blood pressure
- Temperature
- Level of consciousness (Alert, Voice, Pain, Unresponsive – AVPU).

NB – Glasgow Coma Scale (GCS) should be completed at a frequency determined by the specialty team if required and/or SCGOPHCG Falls Management Guideline CPG018

If additional observations are documented on a specialty chart (e.g., Intravenous Analgesia Chart), the minimum physiological observations must still be recorded on the ORC.

## 1.3 Measurement of Observations

Observations must be taken on patients:

- At the time of admission (or initial assessment)
- When a patient is suspected of or at risk of experiencing, an episode of acute deterioration
- At regular intervals throughout the duration of their admission
- Prior to inter and intra hospital transfer
- If requested by patient, or family / carer
- If you are otherwise concerned about a patient

## 1.4 Frequency of Observations

Nursing staff must complete a full set of observations on all inpatients at least every 8 hours.

Based on patient assessment, the minimum frequency of observations may be;

- **decreased** by the Registrar or Consultant from the treating team,
- **decreased** for patients awaiting permanent residential care, permanent aged care residents and end of life palliative patients. These patients, including subacute, should have their observations recorded at a minimum frequency of daily.
- **increased** by nurses, or medical officers based on clinical assessment of a patient's condition.

Changes to the frequency of observations must be clearly documented in the patient's medical record and be consistent with the patients' clinical situation and Goals of Patient Care (GoPC).

## 1.5 Documentation

Medical staff must document the plan for monitoring observations, treatment, and escalation of the deteriorating patient at the time of admission. The management plan must include the frequency of observations, taking into consideration:

- the patient's diagnosis
- presence of comorbidities
- treatment and protocol requirements and restrictions to interventions associated with any Advance Health Directives or Goals of Patient Care forms.

The plan must be reviewed and modified based on the patient's clinical status and / or treatment as clinically appropriate, and the outcomes of this treatment documented in the patient medical record. Any request for review by a Senior Nurse or Medical Officer must be documented on the relevant escalation review sticker and placed in the patient's medical record.

## 2. Escalating Care

### 2.1 Acute physiological deterioration

The SCGOPHCG escalation protocols set out the organisational response required in dealing with different levels of abnormal physiological measurements and observations. The escalation protocols allow for a graded response commensurate with the level of abnormal physiological measurements, changes in physiological measurements or other identified deterioration. It includes appropriate modifications to nursing care, increased monitoring, review by the attending medical officer or team or calling for emergency assistance.<sup>4</sup>

Escalation and MET process details are defined for each SCGOPHCG clinical site providing an emergency service. This provides clinicians with mechanisms to escalate care and call for emergency assistance appropriately.

### 2.2 Acute mental state deterioration

To support staff in identifying and responding to changes in a patient's mental state, SCGOPHCG has provided a recognising and responding to acute Mental State Assessment Descriptions ([Appendix 1](#)).

If new confusion or altered mental state is identified (if unclear whether new or the patient's normal state, it should be assumed to be new until confirmed otherwise) this must be documented and escalated to the Shift Coordinator and treating team without delay.

It is the responsibility of the treating MO to undertake a comprehensive assessment of the patient and to escalate to colleagues with appropriate expertise to manage physical or psychological conditions related to changes in mental state.

The effectiveness of the response must be continuously assessed and adapted to current need. Assess the patient's mental state variation from baseline each shift. The patient's management plan, which must include frequency of observations and monitoring, must be documented in the healthcare record.

Baseline information is an essential component to determining whether deterioration has occurred – this must be established and documented in the healthcare record.

Enquire with the patient, as well as their family, carer and/or Next of Kin (NOK) to determine whether they have observed changes in behaviour consistent with deterioration in the patient's mental state. Families and close friends have a vital role in identifying early stages of deterioration in a person's mental state.<sup>2,3</sup>

If the patient does not exhibit any of the features described above, document their baseline mental state in the healthcare record.<sup>2,3</sup>

- All clinicians will maintain a level of alertness that patients with no identified history of risk may experience deterioration in their mental state, including delirium, when changes in behaviour, cognitive function, perception, physical function, or emotional state are observed or reported.
- Indicators of deteriorating mental health issues may include:
  - reported change
  - distress
  - loss of touch with reality or the consequences of behaviour
  - loss of function
  - elevated risk to self, others, or property
- Responses to deterioration in mental state include, but are not restricted to:
  - increasing the frequency and or level of nursing observations
  - implementing strategies to support and manage behaviours
- Further assessment by specialist mental health clinician may be required.
- The effectiveness of the response must be continuously assessed and adapted to current need.

## **2.3 Physiological deterioration of a mental health patient**

There is overwhelming evidence indicating poor physical health outcomes for people with mental illness.<sup>4</sup> It is important to minimise delays in recognising and escalating a mental health patient's physiological deterioration.

Where a patient's physiological deterioration requires ongoing complex medical care beyond the capacity of a mental health unit, transfer to an appropriate unit or hospital will be considered in accordance with hospital procedures.

## **2.4 Delirium and cognitive impairment**

Cognitive impairment is a broad term which encompasses dementia and delirium, the two most common forms of cognitive impairment. Delirium (also known as acute confusional state) is characterised by an alteration of consciousness with reduced ability to focus, sustain or shift attention, resulting in a cognitive or perceptual disturbance.

Acute deterioration in cognitive state is common in hospitalised patients and may be attributed to physiological compromise or may be a sign of delirium. While delirium can occur in patients of any age, older patients with cognitive impairment, dementia, severe medical illness, or a hip fracture are considered at greatest risk during a hospital admission.



Prevention is the most effective strategy, but outcomes for patients with delirium can also be improved by early recognition and intervention.

Staff should be alert for changes such as disturbance of consciousness, attention, cognition, and perception that develops over a short period of time and acknowledge concerns raised by family members and carers, asking them, “Do you think (name of person) is thinking differently or behaving differently over the last few hours or days?”.

### **3. Modifications and Clinical Management Plan for Altered Parameters**

Modifications to the response criteria can only be made in exceptional circumstances and should only be made by the Registrar / Consultants relevant to the patient at the time. Any modification requires careful consideration, balancing the risk of reduced sensitivity of the escalation system, with the benefit of altered criteria.<sup>1</sup> Modifications can only be made following a full review of the patient, and with sound clinical rationale and justification documented.

#### **3.1 Clinical management plan for altered parameters**

Altered parameters must reflect normal altered physiological parameters and not an acute episode of clinical management. For acceptable adjusted parameter ranges to be valid the Clinical Management Plan for Altered Parameters table must be completed, including parameter range, escalation required along with the rationale, justification and expected outcome of altered intervention to be documented on the ORC.

Altered parameters are only valid for up to 72hrs and then must be reviewed. If on review the clinical rationale and management plan remains unchanged, they can be reindorsed for a further 72hrs by the Consultant or Registrar.

Any patient transferred from Sir Charles Gairdner Hospital (SCGH) may have existing altered parameters in place, which will continue until the treating OPH medical team review the instructions. Changes must be documented by the treating medical team in the patients' medical record and communicated to the nursing team.

#### **3.2 Time limited post procedure modifications**

Time Limited Post Procedure Modifications are only valid for 4 hours after the completion of a procedure or anaesthetic and can only be implemented by the Registrar or Consultant of the procedural team. The clinician completing the time- limited modification is responsible for coordinating follow-up if clinical escalation required.

For adjusted parameter ranges to be valid the Time Limited Post Procedure Modifications table must be completed, including parameter range, escalation required along with the instructions for when observations fall outside of the ranges.

### **4. Responding to deterioration**

Where acute deterioration occurs, it is important to ensure that the capacity exists to obtain appropriate emergency assistance or advice prior to the occurrence of an adverse event such as a cardiac arrest. The emergency assistance provided as part of a Medical Emergency Team (MET) response is additional to the care provided by the attending medical officer or team.<sup>4</sup>



The clinicians providing emergency assistance as part of the response team must:

- be available to respond within agreed timeframes
- be able to assess the patient and provide a provisional diagnosis
- be able to undertake appropriate initial therapeutic intervention
- have authority to make transfer decisions and to access other care providers to deliver definitive care
- seek clinical advice from senior/speciality staff in a timely manner who can provide treatment-limiting decisions
- document events surrounding the actions and treatment recommendations resulting from the call in the patients' medical record.

Early intervention has been shown to reduce mortality and morbidity in the critically ill patient. The decision to initiate a MET call is made in response to established criteria.

### **4.1 Escalation of care**

The escalation protocol refers to the response criteria and the mandatory actions required as indicated on the observation and response chart.

The escalation protocol:

- authorises the clinician to escalate care until they are satisfied an appropriate response has been made. If the delegated clinical staff member is unable to respond to a clinical review request, the calling clinician should escalate to the next escalation level,
- highlights the inclusion of the 'worried' criterion which allows all clinicians to escalate care in the absence of other abnormal observations,
- takes into consideration the concerns of the patient, family, or carers,

The escalation protocol specifies:

- the abnormal physiological parameters (coloured zones),
- the action required for a particular abnormal physiological parameter,
- how the care of the patient is escalated,
- the clinician that the care of the patient is escalated to,
- who has primary responsibility for the escalation response,
- clinician to be contacted when escalation is initiated,
- the timeframe in which a requested response should be provided.

Patients who consecutively trigger during Increased Surveillance or Senior Nurse Review despite interventions should have the response criteria escalated to the next level.

Any patient that meets criteria for escalation to medical review two or more times within a 24-hour period, must be escalated and discussed with the responsible Consultant by the reviewing Medical Officer. The assessment and recommended plan should be clearly documented in the patient's medical record.

In addition to signs of physiological deterioration all clinical staff must initiate a response when they recognise the signs of deterioration in mental state in a person for whom they are providing health care. Key factors in recognising deterioration in mental state may include changes in a person's behaviour, cognitive function, perception, or emotional state.

Medical and Nursing can and should escalate at any time they are concerned over the patients deteriorating condition to treating Consultant. No staff member should be prevented from calling a Medical Emergency if they are concerned about deterioration.

## 4.2 Escalation criteria and action required

### ***Increased Surveillance***

- Record observations at a minimum of 4 hours.
- Inform Shift Coordinator.
- Implement appropriate intervention as prescribed / indicated.

### ***Senior Nurse Review***

- Record observations at least once every 1 hour.
- Senior Nurse (next level to yourself, for e.g., RN to CN, CN to CNS/CNM) **must physically review** the patient in the recommended time frame of 30 minutes and complete the Senior Nurse Review sticker. If SRN unable to attend within 30 minutes escalate to medical review.
- Senior Nurse to consider medical review.
- Patient must be escorted if leaving the ward for procedure / transfer and the escorting nurse must provide a comprehensive clinical handover to receiving department.
- Staff **can and should** escalate to the SRN or After-hours SRN at any time you are concerned or not satisfied with the patients' response to previous interventions.

### ***Medical Review (Registrar / Consultant)***

- Record observations every 15 minutes.
- Complete iSoBAR Medical Review sticker.
- Doctor to physically review patient within 30 minutes. If medical review is not attended within 30 minutes, consider MET call.
- Patient must be escorted if leaving the ward for procedure / transfer and escorting nurse must provide a comprehensive clinical handover to receiving department.

### ***Medical Emergency Team (MET) Response CALL***

- Initiate MET call.
- A patient meeting MET call criterion is not to be transferred between hospitals or within the hospital unless they are assessed, accompanied, and monitored by a clinician with advanced life support skills.

## 5. Patient, Family and Carer Decision-making

Information about observations, deterioration and escalation must be communicated to the patient, family, or carer in a timely and ongoing way.

Shared decision making involves discussion and collaboration between the patient and their clinician. It is about bringing together the patient's values, goals, and preferences with the best available evidence about benefits, risks, and uncertainties of treatment, to reach the most appropriate healthcare decisions for that person.

The Australian Charter of Healthcare Rights (second edition) describes the rights that consumers, or someone they care for, can expect when receiving health care.<sup>2</sup>

Patients have the right to involve the people they want in planning and making decisions about their health care and treatment. This could be a family member, carer, friend, or a consumer advocate such as a social worker. Many health services employ different types of liaison officers, such as Aboriginal Hospital Liaison Officers, who can provide patients with advocacy, information, and support.<sup>2</sup>

## **5.1 Communication with patient, family and or carer**

Clinicians must consider the value of information about potential deterioration from the patient, family, or carer. Information about observations, deterioration and escalation must be communicated to the patient, family, or carer in a timely and ongoing way.

### **5.1.1 Patient/carers treatment preferences and treatment limitations orders**

- Clinicians must address the patient's wishes or preferences for treatment as soon as practicable by ascertaining if the patient has an Advance Health Directive (AHD) or an Advance Care Plan (ACP).
- Efforts should be made to establish if the patient has nominated an Enduring Guardian (EG). EPG is our usual terminology.
- The treating doctor must be informed if one of the above exists and should commence a (GoPC) discussion / form.
- If the patient does not have an AHD or ACP but it is likely that they may clinically deteriorate, they should be offered the opportunity to participate in the development of the GoPC. The patient must be fully informed by the treating doctor about their clinical condition and treatment options. This information and the patient's treatment wishes must be documented to guide future decision making.
- If the patient does not have an AHD or ACP and does not have decision making capacity refer to the DoH Health Consent to Treatment Policy for the decision-making hierarchy in relation to the guardian, family, or carer.
- During the patient's episode of care, preferences for resuscitation and treatment limitations may be applied. This information will be documented on the GoPC form and placed prominently at the front of the medical record.

## **5.2 Older person**

Recognising acute deterioration in older people may be challenging due to complex underlying health issues. Vague and nonspecific complaints are not to be dismissed as they may be signs of acute deterioration. SCGOPHCG requires the screening of all patients over the age of 65 (> 50 for Aboriginal people) on presentation to hospital or changes to their medical state on the nursing admission assessment, and if risk identified on the Behaviour of Concern tool.

An acute deterioration of mental state can be an early marker of a serious condition. Early recognition and escalation can improve the older person's health outcomes and reduce rates of comorbidities for treatable geriatric syndromes.

If a change in the health status occurs in an older person, or changes in vital signs are identified, or if staff are concerned about the person, a senior nurse must undertake full assessment and a medical officer must review the person.

### 5.3 Advance Health Directives (AHD) and Advance Care Plans (ACP)

Clinical staff must discuss the preferences, needs, values, and wishes of the patient at admission or as soon as practicable, referring to valid AHD and ACP where accessible. Clinical staff must adhere to AHDs and ACPs when managing or escalating care of a deteriorating patient.<sup>3</sup>

If the patient does not have decision making capacity clinical staff must refer to the decision-making hierarchy in relation to the guardian, family, or carer.

### 5.4 Goals of Patient Care (GoPC)

All medical staff on the treating team (including interns and resident medical officers) are encouraged to complete Section 1 of the GoPC form. A senior medical officer is responsible for facilitating the GoPC discussion; listening and responding to the patient, family, or carer's questions; initiating timely discussions around treatment options, treatment-limiting orders, and non-beneficial treatments to enable the patient/the person responsible to make an informed GoPC decision; and accurately reflecting the patient's wishes, values, and preferences in the GoPC form.

The GoPC form will guide resuscitation and treatment limitations. They can be extended in their validity for up to a year and clinical staff should check the healthcare record on each admission for a current GoPC form. If a form is valid at the time of the current admission, it should be reviewed to ensure that the goals of care are still current for that admission. If they have changed, then a new form needs to be completed and placed accordingly at the front of the patient's medical record.



### 5.5 Aishwarya's Care Call

Patients, their families, or carers are often aware before clinical staff of subtle changes in their or their loved one's health, but may not feel comfortable, or know how to share their concerns with clinical staff.

Coronial inquests have identified where concerns raised by family members about their loved one's condition were not fully appreciated by clinical staff, leading to tragic outcomes.

Aishwarya's CARE Call provides patients, their families, and carers a way to receive or call for assistance when they feel that the healthcare team has not fully recognised the patient's changing health condition. It supports established SCGOPHCG strategies to improve the recognition and response to clinical deterioration, including the use of SCGOPHCG observation and response charts with defined escalation requirements and site-specific clinical escalation processes.

On admission the patient/carers is to be informed of the (Call and Respond Early) process including:

- Discussing the observation chart and the escalation process
- Providing information on the resources available if they are concerned about the patient's care, including the Aishwarya's CARE Call process.
- Also, how they can contribute to escalating any concerns including:
  - Informing staff of concerns about the patient's clinical condition, treatment, or any changes in the patient's status.
  - How to use the call bell to seek assistance.

The Aishwarya CARE call escalation process comprises three steps:



Reminds the patient, family member or carer to talk to the nurse, midwife or doctor caring for them if they have any concerns about the health of themselves or their loved one's health

Encourages the patient, family member or carer to ask to speak to the shift coordinator, nurse, or midwife in charge if they still have concerns and feel that these have not been addressed by their primary carer.

Provides the ability for the patient, family member or carer to make a CARE call and speak to a senior staff member if they still have concerns that they do not feel have been adequately addressed.

When an Aishwarya's CARE Call is made, a senior staff member should listen to the concerns of the caller, and make a full assessment of the patient's situation, liaising with the treating medical team and other health care providers as required.

The patient/carers/family member who raised the concern must receive feedback after the escalation.

## 6. Medical Emergency Escalation Criteria

Criteria for calling a medical emergency (Code Blue) airway threat, respiratory or cardiac arrest, sudden fall in consciousness, oxygen saturations  $\leq 84\%$ , seizure, unattended medical review or you or a family member are concerned are listed in the escalation protocol on the observation chart and in the Code Blue section of the Emergency Procedures Manual available in all areas and on the SCGOPHCG hub.

Exception: Advance Health Directives; clearly documented 'Not for MET call' or 'Not for Rapid Response' instruction (e.g., GoPC form); 'Not for CPR' or authorised modified observation parameters (e.g., ORC).

If at any time, the escalation process is not progressing in a timely manner, the staff member may contact Executive on Call for advice or assistance.

### 6.1 Medical Emergency Team (MET)

Medical staff on call must have all patients of concern (POC) handed over to them at the commencement of their shift. Clinicians providing emergency assistance must communicate using iSoBAR with the attending medical officer or team.

The consultant with primary responsibility for the care of the patient must be notified by the Medical MET Leader prior to the completion of the MET call regardless of time of day/night. If the primary consultant or their on-call cover is unable to be contacted or make decisions regarding ongoing management, then the MET team will provide emergent care deemed appropriate by the MET team. In this circumstance, at the most appropriate time the Medical Co-Director will be informed.



## 6.2 MET record

The Senior Registered Nurse is to ensure a MET record form is completed for every MET call.

A clinical review should assess and document:

- events preceding the MET
- MET process
- outcome of the MET
- whether a clinical incident has occurred and if investigation has occurred through the Datix Clinical Incident Management System (CIMS).

## 6.3 Roles and responsibilities

| Staff Member                                             | Responsibilities                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |
|----------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Ward Nurse</b><br>(Enrolled Nurse, Registered Nurse,) | <ul style="list-style-type: none"> <li>• Escalate response as per ORC or if concerned</li> <li>• Escalate response if family and or carer indicate concern</li> <li>• Communicate effectively with Shift Coordinator</li> <li>• Document observations and escalation of care appropriately</li> <li>• Ensure resuscitation trolley nearby if deterioration identified</li> <li>• Ensure patient notes and ORCs are available at bedside</li> <li>• Follow instruction by MET leader and deliver appropriate care within their scope of practice</li> <li>• In the case of mental state deterioration, assess the patient and situation, consider escalation to shift coordinator and / or consider calling a code black in case of immediate risk of harm.</li> </ul> |
| <b>Shift Coordinator</b>                                 | <ul style="list-style-type: none"> <li>• Assess patient</li> <li>• Communicate to medical officer if medical review is required</li> <li>• Initiate MET if need identified and if other staff have not already done so</li> <li>• Initiate BLS and ALS (where competent) until MET arrives</li> <li>• Communicate patient's condition, history, and progress to MET</li> <li>• Contact other medical staff as requested by MET leader</li> <li>• In the case of mental state deterioration, assess the patient and situation, consider escalation to medical officer as appropriate and/or consider calling a code black, in case of immediate risk of harm.</li> </ul>                                                                                               |
| <b>Medical Officers</b>                                  | <ul style="list-style-type: none"> <li>• Provide appropriate medical intervention as required</li> <li>• Communicate effectively with the other members of the treating team</li> <li>• Ensure adequate post MET follow up or appropriate escalation of care</li> <li>• In the case of mental state deterioration, assess the patient and situation, consider escalation to mental health treatment team as appropriate and/or consider calling a code black in case of immediate risk of harm.</li> </ul>                                                                                                                                                                                                                                                            |

|                                                                                                                |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
|----------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <p><b>MET Leader</b></p> <p>This role is to be performed by the most experienced clinical expert available</p> | <ul style="list-style-type: none"> <li>• Ensure that it is clear to all team members who the team leader is at the beginning of the medical emergency response and thereafter when this leadership role changes</li> <li>• Be an ALS (adult/paediatric/neonatal as required for the situation) competent and experienced staff member</li> <li>• Communicate effectively with all team members</li> <li>• Identify the tasks or roles required specific to the emergency</li> <li>• Coordinate and delegate roles in accordance with priority of need and in consideration of skill levels/competency/scope of practice of team members</li> <li>• Delegate the retrieval of additional equipment/ medication to an appropriate staff member</li> <li>• Ensure situational awareness of immediate and surrounding environments including the delegation of care of all other patients to appropriate personnel</li> <li>• Maintain a safe working environment</li> </ul> |
| <p><b>Code Black Responders</b></p>                                                                            | <ul style="list-style-type: none"> <li>• Respond to a code black emergency call in accordance with local emergency management protocols</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |

For OPH, after hours, clinical resource personnel are limited. The After Hours CNM may co-opt and deploy one (1) staff member from each area and/or contact on-call staff to assist with managing the immediate emergency and ongoing management. This decision will be made by the After Hours CNM on a case-by-case basis

The CNM and the attending medical officer/s will make the decision whether to manage the patient onsite or transfer to a tertiary facility. This will be based on:

- The stability of the patient
- The availability of appropriately skilled and credentialed staff
- The availability of clinical equipment, medications, and blood products.

## 7. Education and Training

The WA Health MP 0086/18 Recognising and Responding to Acute Deterioration Policy states that Health Service Providers and relevant Contracted Health Entities must ensure that:

- clinical and non-clinical staff are trained, within their scope, to use the systems in place for early recognition of, and response to, acute deterioration
- all staff are to be aware of the local policy, rapid response system and Observation and Response Chart, and know how to activate their local rapid response system.

The NSQHS Recognising and Responding to Acute Deterioration Standard, Action 8.1 outlines that clinicians use the safety and quality systems from the NSQHS Clinical Governance Standard when identifying training requirements for recognising and responding to acute deterioration.



Education for the recognition and response to acute deterioration for medical and nursing staff must be provided at induction and included in mandatory annual refresher training.

It must include:

- systematically assess and evaluate the patient's condition when taking observations
- review vital signs outside of normal range
- initiate appropriate early interventions for patients who are deteriorating and document the intervention/s in the patient medical record
- respond with life-sustaining measures in the event of severe or rapid deterioration, pending the arrival of emergency assistance, and where appropriate contribute to the ongoing acute management of the patient
- immediately communicate all vital signs outside of normal range to other clinicians involved in the care of the patient as outlined within the escalation protocol
- communicate information about clinical deterioration using ISoBAR communication process
- document in the patient medical record all vital signs outside of normal range (along with appropriate interpretation and a treatment plan)
- inform patients, families and carers about the observation monitoring and escalation process
- understand the importance of and discuss end-of-life care planning with the patient, family and/or carer
- undertake tasks required to appropriately care for patients who are deteriorating, such as developing a clinical management plan, adjusting escalation criteria (altered parameters) based on appropriate clinical justification / rationale and documenting clinical deterioration and the response to interventions.

Education for the recognition and response to acute deterioration for allied health staff must include:

- assess a patient appropriate to their professional practice and training
- call for assistance for patients who are deteriorating /appear to be deteriorating
- initiate appropriate early interventions for patients who are deteriorating and document the intervention/s in the patient's medical record respond with life sustaining measures in the event of severe or rapid deterioration, pending the arrival of emergency assistance and, where appropriate contribute to ongoing acute management of the patient as directed.

OPH specific training will also be provided for services or to groups of staff, for specific clinical deterioration scenarios related to their area where there is a high risk of patient mortality and morbidity. Examples include:

- Surgical Services – Massive blood loss
- OPH After Hours Clinical Nurse Managers – Massive blood loss; transfer of patient.

## Code Black Personal Threat

Early response to escalating violence and aggression is an opportunity to better support staff and patients. Where deterioration in mental state (resulting in personal threat, to self or others), an emergency response (Code Black) is activated.

A Code Black Personal Threat activation can be made by any staff member who feels the situation is an emergency and requires assistance and may be called for patients actively attempting suicide or engaging in self-harm.

## 8. Clinical Incident Management

Clinical incidents that are related to a failure or delay in recognising and responding to acute physiological or mental state deterioration must be notified and managed in accordance with the WA Health MP0122/19 Clinical Incident Management Policy 2019.<sup>3</sup>

Staff are required to:

- complete a CIMS Datix notification
- document the clinical incident in the patient's healthcare record.

## Compliance, monitoring and evaluation

The SCGOPHCG Recognising and Responding to Acute Deterioration Committee is responsible for the compliance, monitoring and evaluation of this policy.

Clinical deterioration recognition and response systems are to be routinely evaluated to determine:

- The circumstances and outcome of calls for emergency assistance
- Documentation of core physiological observations
- Compliance with monitoring plans and policies
- Escalation of care,
- Unexpected cardiopulmonary arrests.

Quarterly review of collated analysis of data from MET record forms are to be presented at Committee meetings, with a summary of findings escalated to Clinical Governance

## Related Documents

- [WA DOH Recognising and Responding to Acute Deterioration Guideline](#)
- [Cognitive Impairment Management](#)

Supporting documents

- Observation Response Chart (MR 410/871)
- Goals of Patient Care (GoPC) (MR00H.1)

## References

1. Government of Western Australia East Metropolitan Health Service, Royal Perth Bentley Group. [Recognising and Responding to Acute Deterioration Policy](#). PerthAustralia. December 2020 [Accessed: 7 October 2021]
2. Government of Western Australia South Metropolitan Health Service, Fiona StanleyFremantle Hospital Group. [Mental State Deterioration and Escalation Care Pathway\(Adult\)](#). Perth Australia. September 2021 [Accessed 7 October 2021]

3. Government of Western Australia WA Country Health Service. [Mental Health Care in Emergency Departments and General Wards Policy](#). Perth Australia. September 2018[Accessed 1 December 2021].
4. Stanley, S. & Laugharne, J. (2010). Clinical guidelines for the physical care of mental health Consumers. Community, Culture and Mental Health Unit, School of Psychiatry and Clinical Neurosciences, The University of Western Australia. Perth: The University of Western Australia. Available from: [https://www.chiefpsychiatrist.wa.gov.au/wp-content/uploads/2015/11/Clinical\\_Guidelines\\_Physical\\_Care\\_MH\\_Consumers\\_UWA.pdf](https://www.chiefpsychiatrist.wa.gov.au/wp-content/uploads/2015/11/Clinical_Guidelines_Physical_Care_MH_Consumers_UWA.pdf) [Accessed 18 May 2020]

[https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/NMAHS/SCGH/SCGOPHCG.CPG.Cognitive\\_Impairment\\_Management.pdf](https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/NMAHS/SCGH/SCGOPHCG.CPG.Cognitive_Impairment_Management.pdf)

## Authorisation & Version Control

|                      |                                                                                                                                   |                |            |              |            |
|----------------------|-----------------------------------------------------------------------------------------------------------------------------------|----------------|------------|--------------|------------|
| Policy Sponsor       | Recognising and Responding to Acute Deterioration Committee Chair                                                                 |                |            |              |            |
| Policy Contact       | Recognising and Responding to Acute Deterioration Committee Chair                                                                 |                |            |              |            |
| Date First Issued:   |                                                                                                                                   | Last Reviewed: | 19/03/2023 | Review Date: | 12/05/2026 |
| Version No.          | 5                                                                                                                                 | Reference #:   |            |              |            |
| Approved by:         | Recognising and Responding Acute Deterioration Committee                                                                          |                |            | Date:        | 24/03/2023 |
| Endorsed by:         | SCGOPHCG Policy and Guideline Committee                                                                                           |                |            | Date:        | 04/04/2023 |
| Standards Applicable | Insert relevant NSQHS Standards (Version 2):  |                |            |              |            |

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.

## Appendix 1: Mental State Assessment Descriptions

### 1. Baseline: Past Medical History (PMH)

- Dementia / chronic confusion
- Psych disorder (e.g., psychosis, mood disorder, anxiety)
- Behavioural disorder (e.g., personality disorder, antisocial, OCD)
- Drug / ETOH dependence / abuse
- Cognitive deficit / developmental delay / mental retardation
- NOK / family aware of Mental health (MH) PMH
- Medical team aware of MH PMH on admission

### 2. Changes: Escalation / Deterioration

- Suicidal expressions / ideation / homicidal / Deliberate Self Harm (DSH)
- Hallucinations / delusions / paranoia
- Agitation / restlessness / withdrawn / drowsiness
- Confusion / bizarre behaviour / disoriented
- Other

### 3. Consider possible causes / treatment

- a) Confusion / disorientation / drowsiness (acute / acute on chronic) (esp. in context of abnormal OBS on ORC / fever)
  - Sepsis
  - Intracerebral
  - Medication SE (e.g., opiates, sedatives, anticholinergics) / Medications ceased / recreational drugs / post op
  - Electrolyte imbalance
  - Pain (retention, post procedural, exacerbation of presenting organic illness)
  - Trauma / falls
- b) Confusion / disorientation / drowsiness (chronic)
  - Change to medications
  - Environment (rails, AIN, delirium unit, external stimuli)
- c) Agitation
  - Pain (retention, post procedural, exacerbation of presenting organic illness)
  - Withdrawal
  - Medication SE (e.g., opiates, sedatives, anticholinergics) / Medications ceased / recreational drugs
- d) Mood (elevated or depressed)
  - Extrinsic: Environment / situational crisis / poor diagnosis / cultural or religious needs / language barriers
  - Intrinsic
  - Change to medications
- e) Psychosis
  - Medication SE / Medications ceased / recreational drugs

### 4. Documentation / communication / review response to management

- a) ISOBAR handover
- b) Discuss diagnosis and plan with medical / nursing / patient / NOK
- c) Monitor response to management